

Today's Problems Come From Yesterday's Solutions

Of the eleven laws in Chapter 4, the one that best describes the major issue in healthcare is **“Today's problems come from yesterday's solutions.”** Senge's point is that a problem which appears to arrive from nowhere is usually the downstream effect of a fix made earlier, somewhere else in the system, for reasons that were entirely sound at the time. Because the fix and its consequence are separated by years and by organizational distance, nobody connects the two, and the people now living with the problem are rarely the people who created it.

Dialysis is the clearest case I know. In 1972 Congress extended Medicare to cover end-stage renal disease, which solved an urgent and real problem: people were dying because they could not pay for treatment. The solution paid per treatment delivered in a facility. Fifty years on, the United States has a large, capital-intensive in-center hemodialysis industry; the great majority of patients start in a center, and home modalities — cheaper, better at preserving residual kidney function, and what a substantial share of patients say they would prefer — remain the minority path. The clinic real estate, the staffing ratios, the training pipeline, and the referral habits all grew up around the 1972 payment structure. None of that was anyone's intent. It is yesterday's solution producing today's problem.

The law earns its place because it predicts how the obvious fixes will fail. Exhorting nephrologists to offer home therapy, or bolting a home-education requirement onto the intake visit, treats the symptom and leaves the structure untouched; the structure pushes back, and the home census drifts down again once the initiative's funding ends.

What I would suggest. First, change what is paid for rather than what is asked for: a per-patient, per-month payment that is neutral across modality removes the revenue penalty a program absorbs when a patient goes home. Second, measure the right interval — modality at ninety days and at twelve months, not modality at start — so the metric rewards keeping patients home rather than enrolling them. Third, relieve the constraint that actually binds, which is the supply of home training nurses, not patient interest; funding and credentialing that workforce is unglamorous and is where the leverage sits. Fourth, and most in the spirit of the chapter, make the question part of the decision: before adopting any fix, write down what structure it will create that someone will have to undo in twenty years. Yesterday's solutions were not stupid. They were just never asked that question.